

You selected **3 value drivers**: Hospitalist Retention, Documentation-Related Denials, CC/MCC Capture (DRG Optimization). Each section walks through the logic step by step—what we're measuring, why it matters, and exactly how we calculated it.

Documentation time saved

\$351K

Revenue 71%

Assumes 10% annual value growth with increased adoption

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Hospitalist Retention

\$225K

THE THEORY

Hospitalists spend 2+ hours per day on documentation—much of it after rounds or at home. This drives burnout and turnover. Replacing a hospitalist costs \$400-600K when you factor in recruiting, lost revenue, and onboarding.

The documentation burden is consistently cited as the top complaint among hospitalists considering a change. When that burden lifts, satisfaction improves and turnover drops.

TIMELINE NOTE

Retention impact typically measurable after 12-18 months. Early indicators include satisfaction scores and intent-to-stay surveys.

YOUR CALCULATION

STEP 1: BASELINE TURNOVER

How many hospitalists leave annually?

$$20 \text{ hospitalists} \times 15\% = 3.0 \text{ departures}$$

Hospitalist turnover averages 15-20%. Higher than most specialties due to workload and schedule demands.

STEP 2: BURNOUT-RELATED

How many are tied to burnout?

$$3.0 \text{ departures} \times 50\% = 1.50 \text{ burnout-related}$$

~50% of hospitalist departures cite burnout as a primary factor. Documentation burden is consistently the top complaint.

STEP 3: ABRIDGE ATTRIBUTION

How many can Abridge help prevent?

$$1.50 \text{ at-risk} \times 30\% = 0.45 \text{ prevented}$$

Documentation is a major burnout driver, but not the only one. We conservatively estimate Abridge impacts 30% of burnout-related turnover.

STEP 4: COST SAVINGS

What's the value?

$$0.45 \text{ prevented} \times \$500K = \$225K$$

Hospitalist replacement cost: \$400K-\$600K including recruiting, lost revenue, and onboarding.

Benchmark: Hospitalist Replacement Cost

Recruiting + signing bonus
Lost revenue during vacancy
Onboarding & ramp-up
Total

\$75K - \$150K
\$250K - \$400K
\$50K - \$75K
\$400K - \$600K

Annual Hospitalist Retention Value

0.45 avoided x \$500K

\$225K

What This Means

Over ~2.2 years, expect to retain 1 additional hospitalist you would have otherwise lost to burnout. That's \$225K in avoided replacement costs—not counting the continuity, culture, and quality impacts of physician turnover.

Documentation-Related Denials

\$192K

THE THEORY

Most inpatient denials are appealed due to high stakes—but some are lost forever when documentation can't support the claim. Medical necessity wasn't captured. Status criteria weren't documented.

Abridge captures the clinical reasoning that makes the difference. When the thinking is documented, appeals have substance.

PAYER VARIATION

Denial patterns vary by payer. Work with your revenue cycle team to validate rates for your specific payer mix.

YOUR CALCULATION

STEP 1: TOTAL DENIALS

How many claims are denied annually?

4,875

admissions

x

5%

=

244 denials

Inpatient denial rates typically range 5-10%. Varies by payer mix and case complexity.

STEP 2: DOCUMENTATION-RELATED

How many are caused by documentation gaps?

244

denials

x

35%

=

85 doc-related

35-40% of inpatient denials stem from documentation gaps: medical necessity, level of care, status (IP vs Obs).

STEP 3: WRITTEN OFF

How many are lost without appeal?

85

doc denials

x

25%

=

21 written off

Due to high claim values, most are appealed. But ~25% are ultimately written off—documentation can't support the appeal.

STEP 4: ABRIDGE RECOVERY

How many can Abridge save?

21

written off

x

75%

=

16 recovered

Abridge captures the clinical reasoning and medical necessity that physicians discuss but don't document.

STEP 5: VALUE RECOVERED

What's the dollar impact?

16

claims

x

\$12,000

=

\$192K

Inpatient claims average \$12,000. Range: \$6K-\$50K+ depending on complexity.

Benchmark: Inpatient Claim Value by Complexity

Low complexity admission	\$6,000 - \$10,000
Medium complexity	\$10,000 - \$18,000
High complexity / ICU	\$20,000 - \$50,000+
Blended average	~\$12,000

Annual Documentation-Related Denials Value

16 claims x \$12,000

\$192K

What This Means

16 claims that would have been written off are now recoverable—\$192K in revenue that stays with your organization. Better initial documentation means fewer denials and stronger appeals.

CC/MCC Capture (DRG Optimization)

\$351K

THE THEORY

DRG reimbursement depends on documented comorbidities. Conditions discussed at bedside but not captured in notes mean missed CC/MCC assignments and lower DRG weights.

Abridge ensures what's discussed gets documented. When the clinical picture is complete, coders can assign appropriate complexity—and the DRG reflects the true acuity of care.

VALIDATION REQUIRED

Work with your CDI team to validate capture rates for your specific case mix. Actual opportunity depends on current documentation quality.

YOUR CALCULATION

STEP 1: ADMISSIONS WITH OPPORTUNITY

How many admissions have documentation gaps?

4,875

 admissions x

40%

 =

1,950 opportunities

Studies show 30-50% of admissions have undocumented CC/MCC opportunities. We use 40% as a moderate estimate.

STEP 2: CAPTURE IMPROVEMENT

How many gaps can Abridge help close?

1,950

 gaps x

15%

 =

293 improved

Not every gap is capturable. 15% accounts for cases where Abridge documentation directly enables CC/MCC capture.

STEP 3: DRG WEIGHT IMPACT

What's the gross revenue impact?

293

 improved x

0.4

 DRG lift x

\$6000

 =

\$703K

0.4 DRG weight is a blended average for MCC captures (respiratory failure, sepsis, malnutrition, etc.).

STEP 4: REALITY CHECK

What's the final value after haircuts?

\$703K

 x

50%

 =

\$351K

50% haircut accounts for RAC/PEPPER audits, coder discretion, and cases where documentation doesn't change final code.

Benchmark: What Drives 0.4 DRG Weight?

Acute respiratory failure	+0.3 to +0.5
Sepsis / Severe sepsis	+0.4 to +0.6
Malnutrition	+0.2 to +0.4
Acute encephalopathy	+0.3 to +0.5
Acute kidney injury	+0.1 to +0.3

0.4 is a blended average for MCC captures.

Annual CC/MCC Capture (DRG Optimization) Value

293 improved x 0.4 weight x \$6000 x 50%

\$351K

What This Means

This represents \$351K in additional revenue through more accurate DRG assignment. Work with your CDI team to validate these capture rates for your specific case mix—the actual opportunity may be higher or lower depending on your current documentation quality.

YOUR SCALING JOURNEY

From pilot to full deployment: 4.1x value growth over 24 months

Implementation follows a proven path. Starting with a pilot group of 20 hospitalists, you'll prove value quickly before expanding. This isn't just about adding more hospitalists—it's about building compounding returns as adoption increases.

At full scale with 60 hospitalists and 74% utilization, annual value reaches \$3.15M—a 4.1x increase from pilot. The journey matters as much as the destination.

Value Growth Trajectory



Pilot
\$768K
20 hospitalists



Scaling
\$1.96M
Growing adoption



Full Scale
\$3.15M
60 hospitalists



+ Network
\$3.99M
Compounded

PILOT PHASE

- 20 hospitalists
- 7,500 admissions
- 65% utilization target
- Annual value: **\$768K**

FULL SCALE

- 60 hospitalists
- 74% utilization target
- 24 months timeline
- Annual value: **\$3.15M**

THE COMPOUNDING EFFECT

+\$844K

Additional annual value from network effects at full scale

Value doesn't grow linearly—it compounds. As more hospitalists adopt Abridge, documentation quality improves across the board. CDI teams query less frequently. Denial patterns shift. The value per hospitalist increases even as you add hospitalists.

This is why pilot success often understates full-scale potential. At 74% utilization with 60 hospitalists, network effects add \$844K to your annual value—bringing total potential to \$3.99M.

CONNECTED VALUE

ED + Inpatient compounds your results

When both ED and Inpatient use Abridge, the value compounds. The admission documentation that starts in ED flows directly into inpatient coding, CDI workflows, and denial defense.

DRG Capture

CCs/MCCs documented in ED carry forward—your case mix starts stronger from admission.

CDI Efficiency

When the ED note is complete, CDI teams query less and focus on complex cases.

Denial Prevention

Medical necessity documented at admission is your first line of defense against payer audits.

If you're also using Abridge in ED, the documentation quality benefits below are amplified—you're building on a stronger foundation.

Methodology & Assumptions

YOUR INPUTS

20 hospitalists
7,500 annual admissions
65% utilization rate
2.5 min saved per encounter
\$200/hospitalist/month

INPATIENT BENCHMARKS

Hospitalist replacement: \$400K-\$600K
Base DRG payment: \$6,000
Avg DRG weight lift: 0.4
CDI query rate: 20-40%
Cost per query: \$50-\$100
Denial rate: 5-10%
Avg claim value: \$12,000

CALCULATION PRINCIPLES

Conservative estimates throughout
Medicare rates (not commercial)
Transparent, auditable logic
50% reality check on DRG capture
10% annual growth for Y2-Y3

All benchmarks based on aggregate data from 200+ health system partners. Individual results vary based on patient acuity mix, payer mix, operational factors, and implementation quality. We use conservative assumptions throughout—actual value may be higher.

"Inpatient documentation has the highest stakes—DRGs, CDI, denials, retention all trace back to the note. The difference between a 3x ROI and a 7x ROI usually isn't the technology. It's utilization, workflow integration, and knowing which drivers matter most for your hospitalist program."